

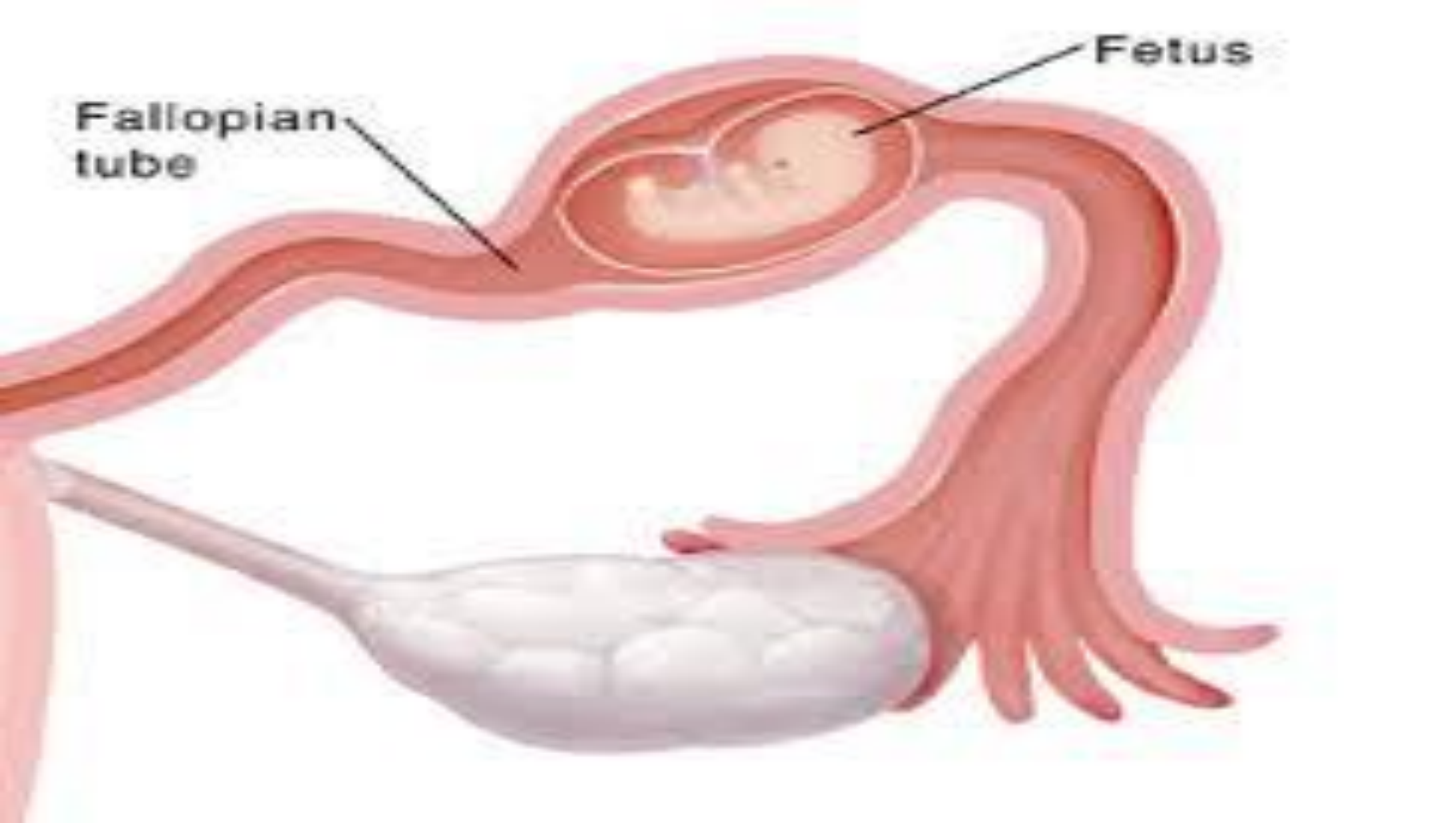
GESTATIONAL PATHOLOGY

BY

MESPA MANYEPA

ECTOPIC PREGNANCY

- Implantation of fertilized ovum at a site other than uterine wall
- Most common site is lumen of fallopian tube
- Key risk factor is scarring



CLASSSSIC PRESENTATION

- Lower quadrant abdominal pain weeks after missed period
- Surgical emergency

SPONTANEOUS ABORTION

- Miscarriage of foetus (20 weeks before gestation)
- Common ; occurs in upto $\frac{1}{4}$ of recognizable pregnancies
- Presents as vaginal bleeding, cramp-like pain and passage of fetal tissue

MOST OFTEN DUE TO CHROMOSOMAL ANOMALIES

- Other causes include hypercoagulable states, congenital infections and exposure to teratogens

PLACENTA PREVIA

- Implantation of the placenta in the lower uterine segment
- Placenta overlies cervical os
- Presents as third- trimester bleeding
- Often requires delivery of fetus by C- sections

PLACENTA ABRUPTION

- Separation of the placenta from decidua prior to delivery of the fetus
- Common cause of still birth
- Presents with third trimester bleeding and fetal insufficiency.

PLACENTA ACCRETA

- Improper implantation of placenta into myometrium with little or no intervening decidua
- Presents with difficult delivery of the placenta and post- partum bleeding
- Often requires hysterectomy

PRE-ECLAMPSIA

- Pregnancy induced HTN, proteinuria and edema; arises in third trimester
- Due to abnormality of maternal- fetal vascular interface in placenta
- HTN can really be severe
- Patient develop fibrin necrosis in the placenta vessels
- Removal of the placenta resolves the problem
- The patient may develop eclampsia
 - Denoted by seizures

HELLP

- Preeclampsia with thrombotic microangiopathy involving liver
- Hemolysis, elevated liver enzyme and low platelets

Sudden infant death syndrome

- Death of health infant (1 month to 1 year) without cause.
- Infants usually expire during sleep
- Risk factor include sleeping on stomach, smoking in household and prematurity

Hydatidiform mole

- Abnormal conception characterized by swollen and edematous villi with proliferation of trophoblasts
- Uterus expands as if normal pregnancy is present

Classically presents in 2nd trimester

- Passage of grape-like masses through vaginal canal

With prenatal care

- Diagnosed by routine ultrasound in early, early first trimester
- Fetal heart sound are absent
- Snow-storm appearance on ultrasound

Classified as complete or partial mole

	Partial mole	Complete mole
genetics	Normal ovum fertilized by two sperm(or one sperm that duplicate chromosomes) 69 chromosomes	Empty ovum fertilized by two sperm(or one sperm that duplicate chromosomes)
Fetal tissue	present	Abscent
Villous edematous	Some villi are hydropic while others are normal	Most villi are hydropic
Trophoblastic proliferation	Focal proliferation are around hydropic villi	Difuse, circumferential proliferation around hydropic villi
Risk for choriocarcinoma	minimal	2- 3 %

Treatment is dilatation and curettage

- B-hCG monitored to ensure adequate mole removal and to screen for development of choriocarcinoma

choriocarcinoma

- May arise as a complication of gestation or as a spontaneous germ cell tumor
- Gestation pathway responds well to chemotherapy
- Germ cell pathway does not

TWALUMBA BASA!

- LWITUMEZI
- ASANTE SANA
- ZIKHOMO